

REGULATORY CHALLENGES FACING PHARMACY PRACTICE IN NIGERIA

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ABSTRACT

Pharmacy is an indispensable component of an effective healthcare delivery system in any country, and Nigeria is no exception. A fundamental responsibility of the Pharmacists Council of Nigeria (PCN) is to ensure the public has access to appropriate pharmaceutical products through registered, inspected, approved, licensed, and monitored premises. These include retail pharmacies, wholesale/importation facilities, hospital pharmacies, and manufacturing plants. This mandate is executed under the authority of the PCN Act 91 of 1992, which empowers the Council to regulate all aspects of pharmacy practice in Nigeria. The legal framework for regulating medicines and pharmaceuticals in the country has evolved through various instruments since 1887.

All pharmaceutical premises in Nigeria must be staffed by qualified and registered pharmacists who are licensed annually by the PCN. The Council's Mandatory Continuing Professional Development (MCPD) programme ensures that only "fit and proper persons" with up-to-date professional competencies are permitted to offer pharmaceutical care to the public in approved settings.

The PCN operates through an elaborate organizational structure within its Registry, designed to implement regulatory mechanisms effectively across the country. Responsibilities are fulfilled at the grassroots level through a network of 35 State Offices and eight Zonal Offices, all reporting to the Registrar, who serves as the Chief Executive Officer.

As a public corporation, the PCN's Governing Council and Chairman are appointed by the President and Commander-in-Chief of the Federal Republic of Nigeria. The Council provides policy direction to the management for the proper control and regulation of pharmacy practice as stipulated in its establishing Act.

This paper enumerates the major challenges facing the implementation of the PCN's statutory mandate. It also briefly describes some of the Council's modest achievements, including its determination to integrate the principles and practice of Knowledge Management (KM) at all functional levels. The bottom line remains that ensuring the Nigerian public has access to licensed pharmaceutical outlets under the supervision of qualified and registered pharmacists will continue to be the PCN's priority.

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1.0 Introduction

Pharmacy practice in Nigeria today is broadly categorized into four specialties:

1. Retail Pharmacy Practice
2. Wholesale/Importation of Pharmaceutical Products
3. Hospital Pharmacy Practice

4. Pharmaceutical Manufacturing

The Pharmacists Council of Nigeria (PCN) is the federal government agency established by Decree 91 (now an Act) of 1992. It is vested with the responsibility of regulating and controlling all aspects of pharmacy practice in Nigeria. This mandate encompasses the registration, inspection, approval, licensing, and monitoring of all premises where pharmaceutical activities occur. Furthermore, the PCN is responsible for regulating the training, registration, and licensing of "fit and proper persons" to study pharmacy and practice the profession in Nigeria.

Therefore, the PCN has the onerous duty of ensuring that only individuals with relevant and appropriate professional competencies are licensed to provide pharmaceutical care to the Nigerian public. These services must be delivered from premises that have been registered, inspected, approved, licensed, and are effectively monitored by the PCN.

The regulatory framework is further defined by other legal instruments, such as:

- The **Inspection, Location and Structure of Pharmaceutical Premises Regulations 2005**
- The **Registration of Pharmaceutical Premises Regulations 2005**

These official gazettes specify critical pharmaceutical powers and limitations.

In 2005 (and revised in 2009), the PCN prepared, edited, and published the **Four-part Compendium of Minimum Standards for the Assurance of Pharmaceutical Care in Nigeria**. This document prescribes the minimum standards required for the four specified areas of pharmacy practice.

Similarly, the PCN developed the **Mandatory Continuing Professional Development (MCPD)** programme, now in its eighth module of the third cycle. The MCPD ensures that pharmacy practitioners remain current in their knowledge within this dynamic profession. Participation in the MCPD and the acquisition of necessary credits are prerequisites for annual licensure to practice pharmacy in Nigeria.

In summary, the PCN ensures that a properly trained individual from an approved institution is matched with an appropriately registered, inspected, approved, licensed, and monitored pharmaceutical premise. This system

certifies that the standards are appropriate for the Nigerian public to access safe pharmaceutical care.

2.0 A Brief History of Pharmacy and Related Laws in Nigeria

The laws controlling pharmacy practice in Nigeria date back to 1878 with the **Lagos Pilotage and Harbour Ordinance**, which regulated the application of medicines and medical treatment for persons on board ships docked in Lagos harbor. This was followed by the **Hospital Ordinance of 1881** and the **Ereko Dispensary Rules of 1889**.

In 1902, **The Pharmacy Ordinance No. 8** was enacted to control the sale and distribution of drugs and poisons, with application restricted to the colony of Lagos and specified protectorate towns. Under this ordinance, dispensers and druggists were examined and granted certificates to practice specific pharmacy-related vocations.

Subsequently, the **Poisons and Pharmacy Ordinance of 1936** was enacted to fortify the registration of dispensers, trained chemists, and druggists. This was substantially amended by the **Poisons and Pharmacy Act Cap 152 of 1958**.

The historical legal framework was definitively reaffirmed by the Nigerian courts. In the case of **Nigerian Association of General Practice Pharmacist Employers (NAGPPE) Vs PCN (Suit No. FHC/KD/CP/8/2001)**, Justice Liman of the Federal High Court, Kaduna, held in 2001:

"I should say with the greatest respect, that both Counsel did not bother to familiarize themselves with the history of pharmacist and pharmaceutical chemists (in Nigeria)... the Pharmacy Act 1958 was... repealed by Section 26(1) of the Pharmacists Council of Nigeria Decree 1992, No. 91... Effectively therefore, the only law regulating the practice of pharmacists in Nigeria today is Decree 91 of 1992." (Emphasis added).

This judicial pronouncement provides a definitive statement on the governing law for pharmacy practice in Nigeria.

3.0 The Registry of the Pharmacists Council of Nigeria

The Head Office and Registry of the PCN is located in the Idu Industrial Area of Abuja, Nigeria's Federal Capital Territory. The Registrar serves as its Chief Executive Officer (CEO).

Since 2004, under the supervision of a 15-member Governing Council appointed by the President and Commander-in-Chief of the Federal Republic of Nigeria, the PCN has initiated a decentralization process. This initiative aims to extend the Council's regulatory and control mechanisms to rural and peripheral areas where the majority of Nigerians reside.

The organizational structure now includes:

- **A Head Office and Registry** in Abuja.
- **Eight Zonal Offices**, each representing one of the country's geopolitical zones: Lagos (South West), Enugu (South East), Uyo (South-South), Minna (North Central), Kaduna (North West), Bauchi (North East), and Abuja.
- **Thirty-five State Offices** operating as 'mini-Registries' across the Federation, supervised by the Zonal Offices.

Each State Office is staffed by at least one pharmacist, assisted by an administrative officer, to provide a basic administrative infrastructure for the PCN's state-wide functions. This structure has significantly improved the quality and speed of PCN's services nationwide.

3.1 Responsibilities of the Pharmacist-in-Charge of State Offices

The Pharmacist-in-Charge of each State Office is responsible for effectively carrying out the following functions:

- i. Processing applications for the renewal of pharmacists' annual licenses to practice.
- ii. Processing applications for the renewal of pharmaceutical premises licenses.
- iii. Coordinating the activities of the State Premises Inspection Committee (PIC).
- iv. Coordinating the activities of the State Patent and Proprietary Medicine Vendors License (PPMVL) Committee.
- v. Supervising and conducting visits to pharmacy internship training centres.
- vi. Participating in routine and new hospital pharmacy inspections with relevant Head Office or Zonal Office staff.
- vii. Participating in inspection visits to new and existing pharmaceutical manufacturing premises with relevant Head Office or Zonal Office staff.
- viii. Participating as an observer in the supervision of Pharmacy Technicians' final examinations alongside Council-approved external examiners.
- ix. Participating in the accreditation of pharmacy programmes in Faculties of

Pharmacy located within the state as part of the PCN Secretariat staff.
x. Participating in the accreditation of Pharmacy Technician programmes at Schools of Health Technology within the state.

xi. Attending the induction and oath-taking ceremonies for new Bachelor of Pharmacy graduates at universities within the state as part of the Registrar's or Zonal Officer's team.

xii. Overseeing the proper implementation and documentation of Mandatory Continuing Professional Development (MCPD) programmes in the state, often organized through the State Branch of the Pharmaceutical Society of Nigeria (PSN).

xiii. Overseeing the proper implementation and documentation of orientation or continuous education programmes for licensed patent medicine vendors in the state.

xiv. Participating as part of the Registrar's team during Orientation Programmes for Foreign-trained Pharmacists at Faculties of Pharmacy within the state.

xv. Receiving all enquiries and complaints from stakeholders and promptly dispatching them to the Registrar through the Zonal Officer-in-Charge. This includes managing the established hotline: 0806 605 5429.

xvi. Ensuring the proper implementation of the State Office's administrative and financial responsibilities in conformity with PCN guidelines.

xvii. Protecting the image and corporate status of the PCN.

xviii. Carrying out any other professional or administrative duties as directed by the Registrar or Zonal Pharmacist-in-Charge (after due clearance with the Registrar).

3.2 Responsibilities of the Pharmacist-in-Charge of Zonal Offices

The eight Zonal Offices supervise the professional and administrative functions of the State Offices within their jurisdiction and report directly to the Registrar. Their responsibilities include:

i. Being directly responsible to the Registrar.

ii. Ensuring the proper implementation of all professional and administrative responsibilities of the PCN in all State Offices within the zone by:

* Maintaining proper and timely contact with PCN staff in State offices to ensure they carry out their functions effectively.

- * Ensuring the availability of Council's inspection vehicles to the States as needed and keeping the Directors of Pharmaceutical Services (DPSs) informed.
 - * Ensuring the availability of appropriate working materials, such as stationery, basic furniture, and equipment in good working order.
 - * Fostering an excellent working relationship between State PCN Offices and State Ministries of Health, particularly the State Pharmaceutical Services Departments.
- iii. Coordinating the incomes and expenditures of all PCN State Offices in the zone on a regular basis and promptly reporting the outcomes to the PCN Head Office/Registry.
- iv. Holding regular follow-up meetings with the DPSs to ensure the timely implementation of:
- * Orientation programmes for licensed patent medicine vendors.
 - * Continuous Education Programmes for eligible patent medicine vendors.
- v. Holding regular follow-up meetings with Deans of Faculties of Pharmacy in the zone regarding the timely implementation of MCPD programmes through the relevant State Offices.
- vi. Liaising with Honourable Commissioners for Health and other relevant State officials to build synergy for effective regulation of the pharmacy profession in accordance with Act 91 of 1992.
- vii. Representing the Registrar or the PCN at social or professional activities in any state within the zone, as directed.
- viii. Submitting separate, quarterly reports to the Registrar on the professional activities of every state in the zone.
- ix. Participating in PCN's periodic Management Committee meetings, as convened with the Registrar's approval.
- x. Convening meetings of the Officers-in-Charge of State PCN Offices, after obtaining necessary clearance and approval from the Registrar.

4.0 Knowledge Management in PCN Functions

The PCN is committed to applying modern Knowledge Management (KM) principles to its core functions—Registration, Control, Monitoring, Regulation, Education, and Training—to enhance the quality of pharmacy practice in Nigeria.

As a first step, the PCN commissioned an internationally renowned Nigerian KM consultancy firm to standardize all its publications. The result has been the creation of three distinct publication series:

- **Registration and Control (ReC) Titles**
- **Monitoring & Regulation (MoR) Titles**
- **Education and Training (EdT) Titles**

Each publication is assigned a unique code within its series, allowing for up to 999 titles per series. This standardization ensures all 20 PCN publications meet UNESCO and international standards. The PCN is proud of this achievement, which positions it as a pacesetter for other Nigerian public corporations in KM.

This logically structured system makes it significantly easier for pharmacy practitioners, related workers, educators, and trainers to search, locate, cite, recall, and purchase PCN publications. A full, classified list of standardized publications to date is shown in **Table 4.1**, with more titles scheduled for future publication.

S/ NO	CODE	FULL TITLE & ISSN
1.	PCN.Rec.001	The establishment of the Pharmacists Council of Nigerian Decree (No. 91 of 1992). Reprinted. 2009. (ISBN: 708-2397-56-3).
2.	PCN.Rec.002	Conditions of Service for Employees of the Pharmacist Council of Nigeria 2 nd ed., 2009. (ISBN: 978-2397-64-4)
3.	PCN.Rec.003	A Compilation of Pharmacy, Drugs and Related Laws and Rules in Nigeria, 1935-2000. 24 ed., 2009. (ISBN: 978-2397-96-2)
4.	PCN.Rec.004	PCN's Approved Patent Medicine List 4 th ed., 2009, (ISBN: 978-2397-72-5)
5.	PCN.Rec.005	Procedures for the Registration of Pharmacist and Pharmaceutical Premises 2 nd ed., 2009 (ISBN: 978-2397-80-6)
6.	PCN.Rec.006	Meeting the Leadership Challenge of a Repositioned PCN, the Council's 2003 – 2007 Scorecard. 2 nd ed., 2009. (ISBN: 978-2397-17)

B. Monitoring and Regulation (MoR) Titles

S/ NO	CODE	FULL TITLE & ISSN
	PCN.MoR.001	PCN's Guidelines for the Issuance of the

		Patent and Proprietary Medicines Vendor's Licence. 24 ed., 2009. (ISBN: 978-2397-25-3)
	PCN.MoR.002	Code of Ethics for Pharmacist in Nigeria. 2 nd ed., 2009. (ISBN: 978-2397-33-4)
	PCN.MoR.003	PCN's Manual for Pharmaceutical Inspectors in Nigeria. 2 ed., 2009. (ISBN: 978-2397-41-5)
	PCN.MoR.004	The PCN Sourcebook on Pharmacy Regulation in Nigeria. 2d ed., 2009 (ISBN: 978-2397-49-0)
	PCN.MoR.005	The four-part compendium of minimum Standards for the Assurance of Pharmaceutical Care in Nigeria. 2d ed., 2009. (Hardback & Paperback editions both in Matt Finis). (ISBN: 708-2397-66-0) (Handback) (ISBN: 987-2397-58-x) (Limp)

b. Education and Training (EdT) Titles

S/No	Code	Full Title & ISSN
12.	PCN.EdT.001	Akubue, Paul I, and Fred B. Adenika (eds). <i>Pharmacy Education in Nigeria: Proceedings of the Pharmacy Curriculum Conference, organized by the Pharmacists Council of Nigeria, August 28- 30, 2001. 24 reprint, 2009.</i> (ISBN: 9782397-65-2).
13	PCN.EdT.002	PCN Lecture Notes for the Orientation Programme of Patent Medicine Vendors in Nigeria. 2d ed., 2009. (ISBN: 978-2397-73-3).
14	PCN.MoR.003	PCN's Guidelines for the Establishments of the Bachelor of Pharmacy Programme in Nigerian Institutions. 3 rd ed., 2009 (ISBN.978-2397-81-4).
15	PCN.MoR.004	PCN's Manual for the Foreign Pharmacy

		Graduate Orientation Programme (FPGOP). 2na ed., 2009. (ISBN: 978-2397-89-X).
16	PCN.MoR.005	PCN's Internship Training Programme for Pharmacy Graduates in Nigeria. 2d ed., 2009. (ISBN: 978-2397-97-0)
17	PCN.MoR.006	PCN's Internship Logbook for Fresh Pharmacy Graduates in Nigeria. 2 nd reprint, 2009. (ISBN:978-2397-18-0)
18	PCN.MoR.007	PCN's Training Manual for Pharmacy Technicians in Nigeria. 3 rd ed., 2009. (ISBN:978-2397-26-1)
19	PCN.MoR.008	PCN's Continuing Education Programme (CEP) for Holders of the Patent and Proprietary Medicines Vendor's Licence (PPMVL) in Nigeria. 2d ed. 2009.
20	PCN.MoR.009	PCN's Mandatory Continuing Professional Development Programme (MCPD) for the Recertification of Pharmacist in Nigeria. 3rd ed., 2009 (ISBN:978-2397-42-3).

5.0 Addressing PCN's Regulatory Challenges

Nigeria is a developing country with a population exceeding 140 million people. It suffers from some of the world's poorest health indices, including high rates of infant and maternal mortality, a high HIV/AIDS prevalence, low life expectancy, endemic malaria, and a resurgence of tuberculosis. These issues are a major concern for all healthcare professionals, both within Nigeria and abroad, including members of associations like NAPPSA.

The country's healthcare infrastructure is inadequate for its population's needs, particularly in rural areas. Health budgets across federal, state, and local government levels, as well as in the private sector, are insufficient and often poorly implemented, limiting their impact. Most critically, there is a severe shortage of trained human resources to manage existing health facilities and to correct the skewed distribution and mix of healthcare professionals across the country.

Pharmacists are in critically short supply. As of December 2008, only 13,775 pharmacists were registered with the PCN, with just 5,489 renewing their annual licenses. This translates to a ratio of approximately one pharmacist for every 10,163 Nigerians, based on a population of 140 million—far below the World Health Organization (WHO) recommended ratio of one pharmacist per 5,000 people. The output of the nine approved Schools of Pharmacy is low,

producing only 667 pharmacists in 2008. While eight more universities have commenced pharmacy programmes at various accreditation stages, the current deficit presents significant regulatory challenges, which can be grouped into six categories:

- i. Inadequate Number of Pharmacists: The shortage of qualified professionals creates a vacuum easily filled by quacks and charlatans, making enforcement of rules and regulations by the limited PCN staff and their Nigeria Police Force collaborators extremely challenging.
- ii. Inadequate Number of Registered Pharmacy Outlets: The scarcity of licensed retail, wholesale, hospital, and manufacturing premises leads to the proliferation of illegal outlets, overstretching the capacity of pharmaceutical inspectors.
- iii. Weak Legal and Punitive Measures: The existing corrective measures often allow offenders within the profession to escape with minor punishments, undermining regulatory authority.
- iv. Insufficient Staff, Volunteers, and Funding: There is an inadequate number of staff and volunteer inspectors, coupled with limited funds, hindering regular and effective inspections necessary to maintain minimum standards.
- v. Need for Enhanced Training: There is an urgent need for regular, effective training for inspectors and PCN staff in modern inspectorate concepts, manufacturing practices, and accreditation standards. Access to overseas training facilities is crucial to ensure inspectors are more knowledgeable than the personnel they assess.
- vi. Weak Cooperation from Stakeholders: Some pharmacists and stakeholders responsible for providing pharmaceutical care exhibit inappropriate or weak cooperation, failing to adhere to stipulated standards. For instance, participation in Mandatory Continuing Professional Development (MCPD) programmes is sometimes met with legal challenges that distract from and negatively impact programme outcomes.

6.0 Concluding Remarks

Benchmarking and standard-setting for the training of pharmacists and pharmacy technicians, and for pharmacy practice itself, constitute the fundamental responsibility of the PCN. The enforcement of these standards is essential to ensure that practice in training institutions and pharmaceutical premises conforms to specified norms. The PCN cannot and will not deviate from this core commitment.

The Council faces an onerous task in fulfilling its statutory mandate. Despite the challenges, its leadership and staff are determined to succeed, bolstered by the goodwill of all stakeholders, including NAPPSA members. This determination and stakeholder support must be maintained and strengthened to protect patients and ensure the safety of pharmaceutical consumers nationwide. If this is done promptly, correctly, and efficiently, the pharmaceutical inspector, as an enforcer of duly constituted regulations, will

have effectively discharged their responsibility to the nation.

May God continue to guide and bless our efforts to move Nigeria forward in this critical endeavour. Thank you for your attention.

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LEARNING OBJECTIVES

Upon completion of this presentation, the audience will be familiar with:

- a. The major provisions of Act 91 of 1992 that establish the PCN as the agency responsible for regulating all aspects of pharmacy practice in Nigeria.
- b. A brief history of pharmacy laws in Nigeria from 1878 to the present.
- c. The legal instruments available for the proper regulation of pharmacy practice.
- d. The managerial structure at PCN Headquarters and its State and Zonal Offices nationwide, designed to ensure pharmaceutical premises are registered, inspected, approved, licensed, and monitored to provide effective care under the supervision of PCN-licensed pharmacists.
- e. The PCN's commitment to applying Knowledge Management (KM) principles, evidenced by the standardization of twenty (20) PCN titles to date.
- f. The key challenges currently facing the regulation and control of pharmacy practice in Nigeria.